consent policy

Version: 1.0 | Effective: | Review:

title: "Consent Policy" effectiveDate: "" reviewDate: "" version: "1.0"

Consent Policy Effective Date: Review Date: Version: 1.0 Applies to: All staff, volunteers, and contractors at [INSERT CARE HOME NAME]

1. Policy Statement At [INSERT CARE HOME NAME], we respect the right of every resident to make informed decisions about their care and treatment. We will always seek valid consent before providing any care, support, or medical treatment. This policy ensures our practices are compliant with: • Mental Capacity Act 2005 • Care Act 2014 • Human Rights Act 1998 • CQC Fundamental Standards – Regulation 11 (Need for Consent)

2. Purpose The purpose of this policy is to: • Ensure consent is obtained before any care or treatment • Support residents to make informed choices • Provide guidance for staff when a person lacks capacity • Promote autonomy, dignity, and human rights

3. Definitions Consent: The voluntary and informed agreement to a proposed intervention by a person with capacity to make that decision. Mental Capacity: The ability to make a specific decision at the time it needs to be made. Assessed in line with the Mental Capacity Act 2005. Best Interests: If a person lacks capacity, decisions must be made in their best interests, using the least restrictive option.

4. Principles of Consent • Consent must be voluntary, informed, and given by someone with capacity. • Consent can be: • Verbal: spoken agreement • Written: signed consent forms • Implied: through actions (e.g., offering an arm for blood pressure check) • Residents can withdraw consent at any time. • All staff must respect a person's right to refuse care, even if this results in risk (unless safeguarding concerns apply).

5. Assessing Capacity Staff must assume capacity unless there is reason to believe otherwise. A capacity assessment must consider: • Can the person understand the information relevant to the decision? • Can they retain that information long enough to make a decision? • Can they use or weigh that information? • Can they communicate their decision (by any means)? If a resident lacks capacity, a best interest decision must be made in line with the Mental Capacity Act 2005.

6. Best Interests Decision-Making When making decisions for someone who lacks capacity, staff must: • Involve the resident as far as possible • Consider their past and present wishes, beliefs, and values • Consult with family, advocates, and professionals • Choose the least restrictive option Where appropriate, an Independent Mental Capacity Advocate (IMCA) must be involved.

7. Consent in Specific Situations Personal Care: • Consent must be gained before delivering any care. • Residents must be offered choices and supported to express preferences. Medication: • Residents must be informed about what the medication is for and side effects. • If a resident refuses medication, this must be respected and documented. Healthcare / Treatment: • Written consent is required for procedures such as vaccinations or dental treatment. • If capacity is lacking, a best interest decision will be made in collaboration with healthcare professionals. Photographs / Recordings: • Explicit written consent is required before taking or using any images or recordings of residents.

8. Recording Consent • All consent (verbal, written or implied) must be documented in the resident's care records. • If consent is refused, this must be recorded along with actions taken and alternatives considered. • Capacity assessments and best interest decisions must be clearly documented.

9. Staff Responsibilities • All staff must receive training on consent and mental capacity. • Staff must know how to assess capacity and follow the Mental Capacity Act Code of Practice. • Staff must not provide care or treatment without appropriate consent or legal authority.

10. Related Policies • Mental Capacity & DoLS Policy • Safeguarding Adults Policy • Data Protection and Confidentiality Policy • Medication Policy • Whistleblowing Policy

11. Monitoring and Review • This policy will be reviewed annually or after any changes in law or guidance. • Regular audits will ensure staff are adhering to consent principles in practice.

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